cynical public relations play with words—to get the "I'm not crazy, just confused" trade. Rather it reflects genuine change in public and professional attitudes, reflecting humane concern and hope for the future of patients, with an increasingly wide range of human problems.

The "Medical Model" of Mental Illness

Along with the new conceptualization of human problems and of modes of mental health care, there is considerable controversy around the "medical or disease model." Continuing to think of psychologically disturbed behavior in disease terms limits attention to essential ethical and social questions and keeps intervention within the institutions of medicine. Psychiatrists, psychologists, sociologists, and social critics have argued that the use of medical metaphors block proper understanding and treatment of psychosocial problems and should be abandoned (Szasz, 1961; Sarbin, 1969; Scheff, 1966, among others). Often these attacks occur from different vantage points, themselves contradictory, and on different issues loosely united within a notion of the "medical model," different aspects of which have been emphasized in each polemic. Here are some of the elements ascribed to a medical conception of psychological dysfunction which have been attacked from one or another viewpoint: (1) There are disease entities, which have etiology, course, and outcome; (2) These diseases are of organic origin; (3) Even if conceived as psychological diseases, they are viewed in analogy with physical ailments. There is an underlying state which is manifested in surface symptoms. Disease is to be inferred from symptoms; changing symptoms will not cure the disease; (4) People get these diseases through no fault of their own; (5) Cure depends on professional intervention, preferably by people with medical training; and (6) The diseases are in the person and, although they may have culturally distinct manifestations, the essential disease process is universal and not culturally specific.

An early and vigorous attack on the "myth of mental illness" was launched by the psychiatrist Thomas Szasz (1960; 1961). By now, he asserted, the concept of mental illness is nothing but a convenient myth. Most mental symptoms cannot be related to lesions of the nervous system. Rather, they are communications the patient offers which characterize his beliefs about himself and the world. They are necessarily parts of his social and ethical context, and should be treated accordingly. The patient should have the right to determine how and why his behavior should change, and he should also be responsible for his acts (see also Mowrer, 1960). Under the guise of medical care deviants are punished and incarcerated. Medicine cannot and should not judge ethical and social issues. And so Szasz argues that psychological disturbances should be ". . . regarded as the expression of man's struggle with the problem of how he should live." And again: "Mental illness is a myth whose function it is to disguise and thus render more palatable the bitter pill of moral conflicts in human relations."

From a quite different vantage point behavioristic clinicians center their attack on the issue of disease and symptom (Krasner and Ullman, 1965). They

argue that the visible symptom is the disease and that there is neither need nor justification for assuming underlying disease states. Their approach to therapy, as we will discuss further in Chapter 14, consists of attempts to remove symptoms or to substitute new behaviors through the application of learning principles. They have less quarrel with other aspects of the medical model. By contrast, existentialistic and humanistic clinicians, at complete variance with the behavioristic group, oppose the deterministic theme in the disease model, and the emphasis on historical and unconscious factors. Far more important, they believe, is the patient's phenomenal experience, his quest for meaning, and his striving to realize his potential. From yet another perspective socially oriented clinicians are unhappy at the focus on the individual patient and on intrapersonal factors, whether organic or psychological, which divert attention from the social systems within which people live and function, well or poorly. Keeping human problems within the scope of medical concepts and institutions, they further argue, limits the innovations (e.g., use of nonprofessionals, new settings, etc.) necessary for comprehensive community care. A more extreme view is represented by those sociological theorists who would see mental illness as another form of social deviance, like criminal behavior, sexual aberrations, immorality, or drug use, which is defined by social values. What is most critical, they contend (e.g., Scheff, 1966) is the valuation society places on the behavior, whether it defines it as sick, dangerous, sinful, or acceptable. In this view, the patient is given a role ("crazy man"), which he can accept and act accordingly. In its full expression, this thesis sees mental disturbance as being determined wholly or primarily by being the recipient and actor of a social role, and in that form it can hardly be accepted. Nonetheless, the importance of labeling and role-ascription as one among other factors which supports disturbed behavior is clearly seen in hospitalized patients, from Pinel to the present (Goffman, 1961). Finally, there are psychologists who contend with the medical model out of status rather than conceptual concerns. Within the medical model, the physician is necessarily in the dominant role while other professions are viewed as ancillary, a term which derives from the Latin for "handmaiden."

The many issues that have emerged in the flow of arguments pro and con are too numerous and complex to review in any detail. Suffice it to note that, in the polemics, medical thinking amd medical practice in areas outside as well as in psychiatry are too often caricatured and straw men are attacked. Particularly as preventative and public health medicine have evolved, with their emphasis on fostering health and development, many of the concepts of modern medicine are quite consonant with the conceptualization of psychological health programs (Dubos, 1959). Moreover, the understanding of psychological functioning has been greatly facilitated by the study of principles of broad generality, such as stress and defense, which figure in biological as well as psychological adaptation. Still, overdependence on medical metaphors for the conceptualization and treatment of psychological problems presents distinct dangers, such as:

1. Both implicitly and explicitly, biological rather than psychological or social factors are stressed. This is true of the patient's conception of his problem, as well as the professional's. "If I have a disease then there must be something wrong with my body. Cut it out or dose it, and I will be well again." Or "If I am sick, what good is it to sit here talking?"

2. The view of the patient as passive recipient of expert care is fostered. It is true of course that in many medical conditions the cooperation of the patient is of

critical importance, but in psychotherapy the patient is, in a significant sense, a coequal partner without whose collaboration therapy is impossible.

3. Medical role models are perpetuated. Too often, these have an authoritarian cast. "The doctor knows best. Follow his prescriptions and you will be well." Needless to say, the "authoritarian doctor and passive patient" relationship can exist if the doctor is a PhD as well as MD. Indeed, psychologically minded psychiatrists, notably psychoanalysts, often go to lengths to dissociate the psychotherapist role from the physician role. The white coat is eschewed for the business suit and the nurse for a receptionist. Thus, the psychological rather than medical aspect of the relationship is emphasized and with it there is greater role equality.

4. The disease model does suggest the importance of medical qualifications. Historically, in the United States, physicians have controlled the mental health field. In all their ramifications, questions of interprofessional relationship, division of labor, the proper training of psychological clinicians, whether in or out of medicine, have given rise to some of the most difficult and exasperating controversies in our field. A psychosocial concept of psychological disturbance does not prejudge the issue and automatically make mental health a problem primarily for physicians. There should be opportunity for qualified professionals of diverse backgrounds to make their best contributions, particularly in a time of manpower shortages in all fields.

5. Emphasis on disease directs attention to a search for specific etiologies, therapies, and prognoses. It can lead to an overemphasis on the diagnostic phase of the clinical process. To my mind, good clinical practice requires clearly describing and understanding a patient's condition (though some clinician might argue the proposition). But overemphasis on diagnosis can divert concern from the ultimate purpose of fostering change.

6. With diagnosis, there are the dangers of labeling and stigmatizing. As noted, calling someone sick rather than bewitched was a historic step forward. But in the present scheme of things a label such as "schizophrenic" may have the effect of casting out the sufferer and consigning him to banishment in a mental hospital. While the label does not create the condition, as some role-theoretical views hold, it does matter greatly. It has social import for the patient, contributes to his self-concept, and the actions of people toward him.

7. The disease model emphasizes the pathological themes in life and neglects the sources of competence and strength in personality development. Professional activity is too often directed toward discovering personality weaknesses rather than strengths. If a person comes to a clinic, he and we view him as ill and being ill needing to be restored to health. The disease model encourages, though by no means does it require, emphasis on pathology.

There is need for vocabulary and ways of thinking which conceives psychological problems in terms other than those traditional in medicine. To argue this point, critics have too often simplified modern medical thinking, though probably not much out of line with common usage. To sophisticated medical thinkers, health is not simply the absence of disease. Thus, the charter of the World Health Organization defines health as "a state of complete physical, mental, and social well-being and not merely the absence of disease or infirmity." Similarly, issues like discrete disease entities, patient passivity, dangers of labeling, doctor authoritarianism, and other matters just discussed have been given thoughtful attention

in medicine, and need not be intrinsic to a "medical model." However, they do represent the kinds of attitudes and distortions which are commonly held and which from the vantage point of psychological health, limit our and the public's understanding of the problem. The words and concepts of medicine, at least as understood by the average man, can block progress. Not unlikely they will remain with us for a good long time but the literal application of medical metaphors should be resisted.